HEALTHCARE

DIAGNOSTIC IMAGING REFERRAL FORM

To Schedule Appointment:
Fax order, notes, demographics to
855-665-6789 or
email: medical@aphealthcare.org

Patient Name Hijarah Hameen Date of Birth 06/24/2001

Patient Phone (803) 669-8510 BILL TO: **APH**

Referring Physician Dr Angela Alvarez D.C Physician Signature Dr. Angela Alvarez, DC Date: 06/12/2024

Reason for Exam Sprain in ligament in cervical and lumbar spine **STAT** ☐ Number to call

Injury Date: : 06/09/2024 Attorney: Law Office Of Jarold A. Boone

Patient Address: 5282 Biffle Downs Road Stone Mountain GA 30088 US

PLEASE NOTIFY TECHNOLOGIST IF YOU ARE OR SUSPECT YOU MIGHT BE PREGNANT.

CIRCLE EXAMINATION DESIRED

HIGH FIELD MRI

☑ **CLOSED** ☐ **OPEN**

☑ **NO CONTRAST** ☐ **WITH/WITHOUT CONTRAST**

- ☐ MR Brain ☐ MR Pituitary
- ☐ MR Temporal Bones
- ☐ MR Angiography ☐ **Brain** ☐ **Neck**
- ☑ MR Cervical Spine
- ☐ MR Thoracic Spine
- ☑ MR Lumbar Spine
- ☐ MR TMJ
- ☐ MR Abdomen LIVER PANCREAS RENAL
- ☐ MRCP
- ☐ MR Pelvis
- ☐ MR Shoulder (R) (L)
- ☐ MR Elbow (R) (L)
- ☐ MR Wrist (R) (L)
- ☐ MR Hand (R) (L)
- ☐ MR Knee (R) (L)
- ☐ MR Hip (R) (L)
- ☐ MR Foot (R) (L)
- ☐ MR Ankle (R) (L)
- ☐ MR Arthrogram - Shoulder - Hip___
- ☐ MRI Enterography
- ☐ MRI Breast Screening / Diagnostic
- ☐ MR OTHER_____

CT SCAN (HELICAL) X - WITHOUT ☐ ORAL

☐ **NO CONTRAST** ☐ **WITH CONTRAST** ☐ **IV**

- ☐ CT Brain
- ☐ CT Angiography ☐ HEAD ☐ NECK
- ☐ CT Sinuses
- ☐ CT Cervical Spine
- ☐ CT Thoracic Spine
- ☐ CT Lumbar Spine
- ☐ CT Calcium Scoring-Coronary Arteries
- ☐ CT Soft Tissue Neck
- ☐ CT Chest
- ☐ CT Abdomen (diaphram to iliaccrest)
- ☐ CT Pelvis
- ☐ CTABD/Pelvis
- ☐ CT Urogram / Stone Protocol (diaphram to pubis)
- ☐ CT Extrem & Reconstructions
- ☐ Low-Dose Lung Screen
- ☐ PE Chest
- ☐ CTA - Runoff
- ☐ CTA Abd/Pelvis
- ☐ CT OTHER_____

NUCLEAR MEDICINE

- ☐ Bone Scan, Limited Whole Body (w/wo Spect)
- ☐ Bone Scan, 3 Phase (w/wo Spect)
- ☐ Gated Blood Pool (MUGA)
- ☐ Hepatobiliary (HIDA) w/cck injection
- ☐ Liver - Spleen Scan w/Spect
- ☐ Octreotide w/Spect
- ☐ Gastric Emptying Scan
- ☐ Gallium for Tumor w/Spect
- ☐ Gallium for Infection
- ☐ WBC Scan/Bone Marrow w/Spect
- ☐ Lung Scan (Ventilation/Perfusion) w/cxr
- ☐ Renal Scan
- ☐ Renal Scan with Lasix
- ☐ Renal Scan with Captopril
- ☐ Thyroid I - 123 Uptake & Scan
- ☐ Thyroid Treatment I -131 (_____mci)
- ☐ Parathyroid w/Spect
- ☐ DAT Scan
- ☐ NM OTHER_____

ULTRASOUND

- ☐ Aorta
- ☐ Abdomen
- ☐ Breast (Please include Mammo film)
- ☐ Carotid Doppler
- ☐ RUQ/Gallbladder/Pancreas/Liver
- ☐ Pelvic Complete
- ☐ Pelvic & Transvaginal
- ☐ Renal
- ☐ Testicular Scrotal with Doppler
- ☐ Venous Doppler, Extremity
  - ☐ Unilateral ☐ Bil.
- ☐ Arterial Doppler, Extremity
  - ☐ Unilateral ☐ Bil.
- ☐ Thyroid
- ☐ Thyroid Biopsy
- ☐ US OTHER_____

SPECIAL PROCEDURES

- ☐ Facet Injection _____
- ☐ Epidural Nerve Root (ESI)_____
- ☐ OTHER _____

FLUOROSCOPY/ABDOMEN

- ☐ IVP (No Tomos)
- ☐ Esophagram/Barium Swallow
- ☐ Upper GI
- ☐ Upper GI & Small Bowel
- ☐ Small Bowel Only
- ☐ Sniff Test

- ☐ **MAMMOGRAM - SCREENING**
- ☐ **MAMMOGRAM - DIAGNOSTIC**
- ☐ **MAMMOGRAM - UNILATERAL** (R) (L)
- ☐ **BONE DENSITY**
- ☐ **INSTANT VERTEBRAL ASSESSMENT-IVA**
- ☐ **Whole Body Composition**

GENERAL X-RAY

- ☐ KUB ☐ Bone Age
- ☐ Abdominal Series
- ☐ Nasal Bones
- ☐ Sinuses
- ☐ Chest PA/LAT
- ☐ Rib/PA Chest (R) (L)
- ☐ Cervical Spine
- ☐ Thoracic Spine
- ☐ Lumbar Spine
- ☐ Pelvis
- ☐ Sacrum/Coccyx
- ☐ Clavicle (R) (L)
- ☐ Shoulder (R) (L)
- ☐ Humerus (R) (L)
- ☐ Elbow (R) (L)
- ☐ Forearm (R) (L)
- ☐ Wrist (R) (L)
- ☐ Hand (R) (L)
- ☐ Hip (R) (L)
- ☐ Femur (R) (L)
- ☐ Tibia/Fibula (R) (L)
- ☐ Knee (R) (L)
- ☐ Ankle (R) (L)
- ☐ Foot (R) (L)
- ☐ Heel (R) (L)
- ☐ SI Joints
- ☐ Scoliosis Series
- ☐ OTHER _____

IF UNABLE TO KEEP APPOINTMENT, PLEASE INFORM US 24 HOURS IN ADVANCE.

EMORY
HEALTHCARE

Fax Transmission

Emory Healthcare
2701 N Decatur Rd Decatur, GA 30033
Phone: 404-501-1000
Fax: 555-555-5555

Date: 6/12/24
Time: 4:27 PM

To: Chair 1

Phone Number:
Fax Number: 470-823-9759

Please find attached a copy of the patient's requested medical records.

Comments: HIJARAH HAMEEN MEDICAL RECORDS

STATEMENT OF CONFIDENTIALITY: This fax message (including any attachments) is for the sole use of the intended recipient(s) and may contain confidential and privileged information. If the reader of this message is not the intended recipient, you are hereby notified that any dissemination, distribution, or copying of this message (including any attachments) is strictly prohibited. If you have received this message in error, please contact the sender and destroy all copies of the original message (including attachments).

06/10/2024 - ED in Emory Hillandale Hospital

ED Provider Note

ED Provider Notes by Lassiter, Richard Gray, MD at 6/10/2024 0005

Patient: HIJARAH HAMEEN

MR# 19195174

Time of Service: 6/10/2024 12:11 AM

History

Chief complaint:
Chief Complaint
Patient presents with
- Motor Vehicle Crash

History of Present Illness

Hijarah Hameen is a 22 y.o. female with a history of no medical problems who presents to the emergency department with complaints of MVC. Patient reports that she was restrained driver and she was rear-ended. She reports that she subsequently struck the car in front of her. She is complaining primarily of neck and back pain after the accident. Does not have any neurological deficits. Has not take any medications. Reports the car was totaled.

History reviewed. No pertinent past medical history.
History reviewed. No pertinent surgical history.
No family history on file.
Social History

Socioeconomic History
- Marital status: Single

Tobacco Use
- Smoking status: Never
- Smokeless tobacco: Never

Substance and Sexual Activity

Confidentiality Statement: This information is subject to all federal and state confidentiality/privacy laws and Emory Healthcare confidentiality/privacy policies. Any unauthorized use, disclosure, or reproduction of this information is strictly prohibited.

- Alcohol use: Not Currently
- Drug use: Not Currently

Social Determinants of Health

Social Connections: Unknown (8/16/2021)
Received from Prisma Health
Social Connections
- Frequency of Communication with Friends and Family: Not asked
- Frequency of Social Gatherings with Friends and Family: Not asked

Intimate Partner Violence: Unknown (8/16/2021)
Received from Prisma Health
Intimate Partner Violence
- Fear of Current or Ex-Partner: Not asked
- Emotionally Abused: Not asked
- Physically Abused: Not asked
- Sexually Abused: Not asked

Housing Stability: Not At Risk (3/10/2022)
Received from Prisma Health
Housing Stability
- Was there a time when you did not have a steady place to sleep: Not asked
- Worried that the place you are staying is making you sick: Not asked

Current Medications:
Reviewed in chart.

Allergies:

Patient has no known allergies.

Review of Systems

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Pertinent positives and negatives as per HPI, otherwise all other systems reviewed and otherwise negative.

Physical Exam

BP 145/90 | Pulse 65 | Temp 36.8 °C (98.2 °F) (Oral) | Resp 17 | Ht 1.6 m (5' 3") | Wt 63.5 kg (140 lb) | SpO2 100% | BMI 24.80 kg/m^2

Pulse Oxygen Saturation was interpreted by me as normal.

Vitals and nursing note reviewed by myself.

Physical Exam
Vitals and nursing note reviewed.
Constitutional:
  Appearance: Normal appearance.
HENT:
  Head: Normocephalic.
Cardiovascular:
  Rate and Rhythm: Normal rate and regular rhythm.
  Comments: **Normal perfusion**
Pulmonary:
  Effort: Pulmonary effort is normal.
Abdominal:
  General: There is no distension.
  Palpations: Abdomen is soft. There is no mass.
  Tenderness: There is no abdominal tenderness.
Musculoskeletal:
  General: Tenderness **(ttp entire cervical spine, mild tenderness to the para spinal musculature. Tender along the thoracic spine as well.)** present. Normal range of motion.
  Cervical back: Neck supple.
Skin:
  General: Skin is warm and dry.

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Comments: **No seatbelt marks or bruises on the chest or abdomen.**

Neurological:

General: No focal deficit present.

Mental Status: She is alert. Mental status is at baseline.

Comments: **Normal strength and sensation of the bilateral upper and lower extremities.**

Psychiatric:

Mood and Affect: Mood normal.

Diagnostics

No results found for this or any previous visit (from the past 4464 hour(s)).
An ECG was not performed on this patient

Laboratory:
Labs studies were ordered and reviewed by myself.

Labs Reviewed - No data to display

Radiography/Imaging

Radiographic studies ordered and reviewed by myself.

CT Thoracic Spine WO Contrast
Final Result
No acute fracture or malalignment of the thoracic spine.

Electronically Signed By: Steadman, Brent On 6/10/2024 02:51:15

Confidentiality Statement: This information is subject to all federal and state confidentiality/privacy laws and Emory Healthcare confidentiality/privacy policies. Any unauthorized use, disclosure, or reproduction of this information is strictly prohibited.

CT Cervical Spine WO Contrast
Final Result
No acute fracture or malalignment of the spine.

Electronically Signed By: Steadman, Brent On
6/10/2024 02:48:13

MDM

Assessment and Plan:

Hijarah Hameen is a 22 y.o. female who presents with after motor vehicle collision. Patient has physical exam that is consistent with muscle pain after a car accident. There is no bony tenderness palpation. Mechanism of injury is consistent with location of the pains. Based on location of her pain, I did order CT scan of the cervical and thoracic spine. These are reviewed by the radiologist and negative.
I have subsequently clear the patient's C-spine and remove her c-collar.
I think this can be appropriately treated with muscle relaxers and NSAIDs.

Today I am discharging the patient with prescriptions for NSAID and Muscle Relaxer. Advised patient to treat the muscle spasm with heat and massage as well.

ED Course

ED Course as of 06/10/24 0430
Lassiter, Richard Gray's Documentation
Mon Jun 10, 2024
0326 CT Cervical Spine WO Contrast
neg
0326 CT Thoracic Spine WO Contrast
neg

Confidentiality Statement: This information is subject to all federal and state confidentiality/privacy laws and Emory Healthcare confidentiality/privacy policies. Any unauthorized use, disclosure, or reproduction of this information is strictly prohibited.

Diagnoses as of 06/10/24 0430
Motor vehicle accident, initial encounter
Cervical strain, acute, initial encounter

Meds administered in the ED:

Medications
ibuprofen tablet 800 mg (800 mg oral Given 6/10/24 0151)

Disposition

Stabilized/Treated and Discharged

I discussed with the patient and/or family about the patient's care plan. All available questions answered.

Consults:
None required

Prescriptions for the patient:
Discharge Medication List as of 6/10/2024 3:56 AM

START taking these medications

	Details
cyclobenzaprine (Flexeril) 10 mg tablet	Take 1 tablet (10 mg) by mouth if needed in the morning and at bedtime for muscle spasms for up to 10 days., Starting Mon 6/10/2024, Until Thu 6/20/2024 at 2359, Print
ibuprofen 600 mg tablet	Take 1 tablet (600 mg) by mouth every 6 (six) hours if needed for mild pain for up to 7 days., Starting Mon 6/10/2024, Until Mon 6/17/2024 at 2359, Print

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ED Provider Note (continued)

99285 - Consider this chart for level 5 billing for the high complexity of medical decision making and consideration of the extensive amount of data reviewed along with the high risk of complications of the differential diagnosis.

Procedures

DIAGNOSIS

1. **Motor vehicle accident, initial encounter**
2. Cervical strain, acute, initial encounter

Portions of this record were created using voice recognition software. Occasional normal work or sound-alike substitutions may have occurred due to the inherent limitations of voice recognition software. Please read the chart carefully and recognize using context where substitutions have occurred.

Electronically signed by

Lassiter, Richard Gray, MD
06/10/24 0430

Electronically signed by Lassiter, Richard Gray, MD at 6/10/2024 4:30 AM

Imaging

Confidentiality Statement: This information is subject to all federal and state confidentiality/privacy laws and Emory Healthcare confidentiality/privacy policies. Any unauthorized use, disclosure, or reproduction of this information is strictly prohibited.

06/10/2024 - ED in Emory Hillandale Hospital (continued)

Imaging (continued)

Imaging

CT Thoracic Spine WO Contrast (Final result)

CT Thoracic Spine WO Contrast Resulted: 06/10/24 0251, Result status: Final result

Ordering provider: Lassiter, Richard Gray, MD 06/10/24 0143
Order status: Completed
Resulted by: Steadman, Brent Thomas, MD
Filed by: Interface, Radiology Results In 06/10/24 0251
Performed: 06/10/24 0216 - 06/10/24 0230
Accession number: E13047198
Resulting lab: IMAGING
Narrative:
PROCEDURE INFORMATION:
Exam: CT Thoracic Spine Without Contrast
Exam date and time: 6/10/2024 2:17 AM
Age: 22 years old
Clinical indication: Injury or trauma; Auto accident; Sprain or strain;
Additional info: MVC back pain

TECHNIQUE:
Imaging protocol: Computed tomography of the thoracic spine without contrast.
Radiation optimization: All CT scans at this facility use at least one of these dose optimization techniques: automated exposure control; mA and/or kV adjustment per patient size (includes targeted exams where dose is matched to clinical indication); or iterative reconstruction.

COMPARISON:
CT CERVICAL SPINE WO CONTRAST 6/10/2024 2:17 AM

FINDINGS:
Bones/joints: No acute fracture or malalignment.

Soft tissues: Unremarkable.

Impression:
No acute fracture or malalignment of the thoracic spine.

Electronically Signed By: Steadman, Brent On 6/10/2024 02:51:15

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
22 - IMG	IMAGING	Unknown	Unknown	05/21/21 1442 - Present

CT Cervical Spine WO Contrast (Final result)

Confidentiality Statement: This information is subject to all federal and state confidentiality/privacy laws and Emory Healthcare confidentiality/privacy policies. Any unauthorized use, disclosure, or reproduction of this information is strictly prohibited.

Imaging (continued)

CT Cervical Spine WO Contrast Resulted: 06/10/24 0248, Result status: Final result

Ordering provider: Lassiter, Richard Gray, MD 06/10/24 0143
Resulted by: Steadman, Brent Thomas, MD
Performed: 06/10/24 0216 - 06/10/24 0230
Resulting lab: IMAGING
Order status: Completed
Filed by: Interface, Radiology Results In 06/10/24 0248
Accession number: E13047197

Narrative:
PROCEDURE INFORMATION:
Exam: CT Cervical Spine Without Contrast
Exam date and time: 6/10/2024 2:17 AM
Age: 22 years old
Clinical indication: Injury or trauma; Auto accident; Sprain or strain, cervical ligaments; Additional info: MVC neck pain

TECHNIQUE:
Imaging protocol: Computed tomography of the cervical spine without contrast.
Radiation optimization: All CT scans at this facility use at least one of these dose optimization techniques: automated exposure control; mA and/or kV adjustment per patient size (includes targeted exams where dose is matched to clinical indication); or iterative reconstruction.

COMPARISON:
CT THORACIC SPINE WO CONTRAST 6/10/2024 2:17 AM

FINDINGS:
Bones: No acute fracture or malalignment of the spine.

Lungs: Lung apices are normal.

Soft tissues: Unremarkable.

Impression:
No acute fracture or malalignment of the spine.

Electronically Signed By: Steadman, Brent On 6/10/2024 02:48:13

Testing Performed By

Lab - Abbreviation	Name	Director	Address	Valid Date Range
22 - IMG	IMAGING	Unknown	Unknown	05/21/21 1442 - Present

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END OF REPORT

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1458 Church St. Ste D
Decatur, GA 30033
Phone: 404.937.2965 | Fax: 404.745.0354

3T High-Field MRI, Upright Open MRI, X-Ray

Patient Name:	Hijarah Hameen	**Referring Physician:**	Angela Alvarez
Patient ID:	1107290	**Referring Phone:**	(404) 257-6777
Patient DOB:	06/24/2001	**Referring Fax:**	(470) 823-9759
DOS:	06/13/2024		

EXAMINATION: MRI LUMBAR WITHOUT CONTRAST

INDICATION: The patient complains of low-back pain subsequent to motor-vehicle accident on 06/08/2024 with new onset of low-back pain (M54.5).

TECHNIQUE: An MRI of the lumbar spine was obtained using sagittal T1 and T2-weighted images. A set of para-axial T2-weighted images was acquired through the lumbar spine intervertebral discs without the administration of gadolinium contrast. No previous studies or reports of previous studies were available for comparison or correlation at the time of the interpretation.

FINDINGS: The conus medullaris is normal in signal and morphology and terminates at the level of the body of L1. The paravertebral musculature is normal in MRI appearance. The visualized portions of the abdominal aorta are normal caliber.

At L5-S1, there is no neural foraminal narrowing or central spinal canal stenosis.

At L4-L5, there is posterior bulge of the disc with bilateral neural foraminal narrowing.

At L3-L4, there is posterior bulge of the disc with bilateral neural foraminal narrowing.

At L2-L3, there is no neural foraminal narrowing or central spinal canal stenosis.

At L1-L2, there is no neural foraminal narrowing or central spinal canal stenosis.

At T12-L1, there is no neural foraminal narrowing or central spinal canal stenosis.

IMPRESSION:

AT L4-L5, THERE IS POSTERIOR BULGE OF THE DISC WITH BILATERAL NEURAL FORAMINAL NARROWING.

AT L3-L4, THERE IS POSTERIOR BULGE OF THE DISC WITH BILATERAL NEURAL FORAMINAL NARROWING.

NO ACUTE FRACTURE OF THE LUMBAR SPINE IS IDENTIFIED.

Doug Hornsby MD

doughornsby@msn.com

Electronically signed on: 06/17/2024 15:18
Transcribed by: Velazquez, Grace on: 06/17/2024 12:44

1458 Church St. Ste D
Decatur, GA 30033
Phone: 404.937.2965 | Fax: 404.745.0354

3T High-Field MRI, Upright Open MRI, X-Ray

Patient Name:	Hijarah Hameen	**Referring Physician:**	Angela Alvarez
Patient ID:	1107290	**Referring Phone:**	(404) 257-6777
Patient DOB:	06/24/2001	**Referring Fax:**	(470) 823-9759
DOS:	06/13/2024		

EXAMINATION: MRI CERVICAL WITHOUT CONTRAST

INDICATION: The patient has a history of neck pain subsequent to motor-vehicle accident on 06/08/2024. ICD-10 code is M54.2. No history of related surgery is provided.

TECHNIQUE: An MRI of the cervical spine is obtained using sagittal T1 and T2-weighted images as well as a set of para-axial T2-weighted images through the intervertebral discs of the cervical spine without the administration of gadolinium contrast. There are no previous studies or reports of previous studies available for comparison or correlation at the time of the interpretation.

FINDINGS: The cerebellar tonsils have a normal anatomic relationship with the foramen magnum. There is straightening of the cervical spine. The odontoid is intact.

At C2-C3, there is no neural foraminal narrowing or central spinal canal stenosis.

At C3-C4, there is posterior bulge of the disc without stenosis.

At C4-C5, there is posterior bulge of the disc effacing the ventral aspect of the thecal sac.

At C5-C6, there is posterior bulge of the disc effacing the ventral aspect of the thecal sac posterolateral to the right.

At C6-C7, there is no neural foraminal narrowing or central spinal canal stenosis.

At C7-T1, there is no neural foraminal narrowing or central spinal canal stenosis.

The cervical spinal cord is normal in MRI signal and morphology.

IMPRESSION:

AT C3-C4, THERE IS POSTERIOR BULGE OF THE DISC WITHOUT STENOSIS.

AT C4-C5, THERE IS POSTERIOR BULGE OF THE DISC EFFACING THE VENTRAL ASPECT OF THE THECAL SAC.

AT C5-C6, THERE IS POSTERIOR BULGE OF THE DISC EFFACING THE VENTRAL ASPECT OF THE THECAL SAC POSTEROLATERAL TO THE RIGHT.

MARKED STRAIGHTENING OF THE CERVICAL SPINE WHICH MAY BE DUE TO STRAIN AND/OR ALTERED ANATOMY.

NO ACUTE FRACTURE OF THE CERVICAL SPINE IS IDENTIFIED.

_____
Doug Hornsby MD

1458 Church St. Ste D
Decatur, GA 30033
Phone: 404.937.2965 | Fax: 404.745.0354

3T High-Field MRI, Upright Open MRI, X-Ray

Patient Name:	Hijarah Hameen	**Referring Physician:**	Angela Alvarez
Patient ID:	1107290	**Referring Phone:**	(404) 257-6777
Patient DOB:	06/24/2001	**Referring Fax:**	(470) 823-9759
DOS:	06/13/2024		

EXAMINATION: **MRI CERVICAL WITHOUT CONTRAST**

doughornsby@msn.com

Electronically signed on: 06/17/2024 09:58
Transcribed by: Velazquez, Grace on: 06/17/2024 09:28

Phoenix Ortho Injury Spine and Extremity

Performing Provider: Rachel Renfro

Hijarah Hameen, 23 yrs Female, **DOB**: 06/24/2001
Visit Date: 07/02/2024 01:30PM
Visit Location: MAIN OFFICE, 1805 PARKE PLAZA CIR STE 101, STONE MOUNTAIN, GA 30087-3646
Address: 5282 Biffle Downs Rd, Stone mountain, Georgia 30088-3829

Phone (Primary): 803-669-8510
Phone (Secondary): n/a
Email: n/a
NOV: New patient
SSN: n/a
Insurance: Self

Intake

Allergies

No known active allergies

Medications

No known active medications

Problems

No known active problems

Vital Signs

No Vital Signs captured during this encounter

History of Present Illness

The patient presents today to establish care following an MVA on 06/09/2024. She indicates she was the driver of a vehicle that was rear-ended. She denies loss of consciousness, was restrained, airbags did not deploy. She was treated and released at Emory Hillendale. She endorses neck pain that radiates into bilateral shoulders and into the midback. She denies numbness/tingling/ weakness. She denies headaches, blurred vision, or dizziness. She has completed cervical and lumbar MRIs. She has been going to chiropractic care for the last month and has seen 80% progressive improvement with this.

Review of Systems

Encounter Background Information: all other systems reviewed and negative;

Musculoskeletal Symptoms: muscle aches; **muscle cramps**; *neck pain*

Physical Exam

General Appearance: oriented to time, place, and person; well developed; well nourished;

Eye Exam: no periorbital swelling; conjunctiva normal;

Lung Exam: unlabored respiration;

Cervical Spine no range of motion decreased; **muscle spasm at trigger point of cervical muscles** ; **pain with lifting or after work, not elicited on exam**;

Imaging

Cervical MRI

C3-4 and C4-5 disc bulges

Lumbar MRI

L3-4 and L4-5 disc bulges

Assessment and Plan

1. **Bulging lumbar disc M51.36 (722.52):**
   07/02/2024

2. **Bulging of cervical intervertebral disc M50.30 (722.4):**
   07/02/2024

Plan

The patient presents today to establish care following an MVA. We reviewed her MRIs. She has seen 80% improvement with chiropractic care. As her neck pain does continue to be intermittently bothersome we discussed the possibility of steroid versus platelet-rich plasma as an epidural injection. She would like to consider this. She is completing chiropractic care next week. I would like for her to complete this and return to normal level of function to assess for acute flare-ups. Follow up in 3 weeks.

Electronically Signed By Rachel Renfro on 07/02/24 at 03:06 PM EDT

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

Initial Exam

Patient Name: Hijarah Hameen
Date of Birth: 6/24/2001

Provider: Angela Alvarez, DC
Date of Service: 6/11/2024

SUBJECTIVE

HISTORY OF PRESENT ILLNESS

She has never experienced this condition before.

PRIMARY COMPLAINT

Neck

- Aching pain: 8/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

SECOND COMPLAINT

Left scapula, Right scapula, Upper back

- Aching pain: 7/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

THIRD COMPLAINT

Mid back

- Aching pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT

Low back

- Aching pain: 5/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

PAST, FAMILY, SOCIAL HISTORY

Surgeries/Hospitalizations: none reported.
Allergies: none reported.

Medications: Ibuprofen and a muscle relaxant.
Prior conditions: none reported.

Mother: Arthritis
Retail manager , single, no children. Pt reports does not know complete Family Health History.

REVIEW OF SYSTEMS

ALLERGIC-IMMUNOLOGIC: Catch colds easily, Frequent sinus trouble and Fever
CARDIOVASCULAR: Chest pain, Dizziness, Shortness of breath and Pressure over the chest
CONSTITUTIONAL: Fatigue and Fever
EAR/NOSE/THROAT: Difficulty hearing, Ringing in ears, Sinus trouble, Nasal stuffiness, Hearing loss and Ear pain
EYES: Glasses/Contacts
GASTROINTESTINAL: Heartburn/Reflux, Nausea/Vomiting and Diarrhea
MUSCULOSKELETAL: Joint Pain/Swelling, Stiffness, Muscle pain, Neck pain, Stiff neck, Back pain and Muscle weakness
NEUROLOGICAL: Loss of strength, Fainting and Light Headed/Dizzy
RESPIRATORY: Cough and Chest pain
SKIN: Rash/Sores and Itching/Burning
WOMEN'S HEALTH ISSUES: Vaginal discharge
GENERAL: Recent weight loss and Fatigue

The following are negative: ENDOCRINE, GENITOURINARY, HEMATOLOGY/LYMPH and PSYCHIATRIC.

Collision occurred on 6/9/2024 at approximately 10:45 PM.
On the highway, rear ended
Symptoms reported: Neck, Chest, and Back Pain, has muscle stiffness all over
Reports may have had CT scans at the hospital.

Ms. Hameen was the driver of the vehicle, a 2010 Honda Civic, at the time of the collision. There was rear, front, bumper, fender damage to the vehicle. The collision happened while she was slowly moving at I-20 in Decatur , GA and occurred at night. The weather was clear. Street surface was dry. The impact on the vehicle was at the rear end. Upon impact, the brakes were loosely applied, and the car moved 1 to 5 feet. Ms. Hameen was seated in the vehicle as a driver and was wearing a seat belt with the shoulder harness and with the headrest in up position. The vehicle is equipped with air bags and they did not deploy. Ms. Hameen didn't see the impact coming. Upon impact, her head was looking down and she was thrown forward. Her chest hit collar bone. Ms. Hameen remembers the collision happening.
She was taken by ambulance and remained in Hillandale Emory for 4 hours
* Medications given: Ibuprofen and a muscle relaxant .

The patient reports to this office due to injuries received in an MVC The patient had prior treatment. Will request records from the other health care providers.

OBJECTIVE

VITALS

Height: 5’3 in Weight: 140 lbs
Temperature: 97.0°F Pulse rate: 66 BPM (relaxed)

Blood Pressure (seated): right -N/A left-85/56 mmHg
Demeanor: coherent and relaxed.

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T10; T11; T12; L1; L2; L3; L4; L5.

Tenderness

Left Side

- Cervical Spine (moderate)
- Thoracic Spine (moderate-severe)
- Lower Back (moderate-severe)

Right Side

- Cervical Spine (moderate)
- Thoracic Spine (moderate-severe)
- Lower Back (moderate-severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (moderate-severe)
- Thoracic Spine (moderate-severe)
- Lower Back (moderate-severe)

Right Side

- Cervical Spine (moderate-severe)
- Thoracic Spine (moderate-severe)
- Lower Back (moderate-severe)

GAIT

Ambulation: Pain.
Gait: Walk Toe out.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Test	Left	Right	Comment
Maximal foraminal compression test[1]	Positive	Positive	
Shoulder depression test[2]	Positive	Positive	
Soto-Hall[3]	Positive	Positive	
Spinal percussion test-cervical[4]	Positive	Positive	
Schepelmann's sign[5]	Positive	Positive	
Spinal percussion test-thoracic[6]	Positive	Positive	
Demianoff sign test[7]	Positive	Positive	
Milgram's test[8]	Positive	Positive	
Spinal percussion test-lumbar[9]	Positive	Positive	

[1]Increased peripheral pain could indicate radiculopathy or IVF encroachment. Local neck pain could indicate joint or muscle damage.

[2]Pain in the arm could indicate adhesions of the nerve roots. Local pain could indicate sprain or strain or adhesions of the shoulder joint capsule.

[3]Radicular pain could indicate nerve root tension. Local pain may indicate sprain, strain, or rib dysfunction.

[4]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

[5]Pain on the concave side may indicate intercostal neuritis. Pain on the convex side may be due to intercostal myofascitis.

[6]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

[7]Perhipheral pain is suggestive of nerve root irritation, such as that due to a herniated disc.

[8]Lower back pain may indicate a herniated disc.

[9]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

REFLEXES

Biceps and Brachioradialis are within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	25° (50% of normal)	Pain, Spasm, Guarding	
Extension	35° (58% of normal)	Pain, Spasm, Guarding	
Left lateral flexion	30° (66% of normal)	Pain, Spasm, Guarding	
Right lateral flexion	25° (55% of normal)	Pain, Spasm, Guarding	
Left rotation	40° (50% of normal)	Pain, Spasm, Guarding	
Right rotation	50° (62% of normal)	Pain, Spasm, Guarding	

Lumbar ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	30° (50% of normal)	Pain, Spasm, Guarding	
Extension	10° (40% of normal)	Pain, Spasm, Guarding	
Left lateral flexion	20° (80% of normal)	Pain, Spasm, Guarding	
Right ateral flexion	15° (60% of normal)	Pain, Spasm, Guarding	
Left rotation	15° (50% of normal)	Pain, Spasm, Guarding	
Right rotation	20° (66% of normal)	Pain, Spasm, Guarding	

DERMATOMES

Posterior

Left-Cervical, Right-Cervical, Left-Lumbar and Right-Lumbar are within normal limits.

PHYSICAL EXAMINATION

General [well nourished, no acute distress]: WNL.

Skin [no skin sensitivity reported, free of lesions, rashes, eczema, upper/lower extremities]: WNL.

Head [eyes – symmetric appearance, conjunctiva white, no discharge, erythema, lid lag or swelling]: WNL.

Ears [no discharge or deformities noted to external ears]: WNL.

Nose [no sinus tenderness, epistaxis, or discharge]: WNL.
Oropharynx [lips free of lesions, cyanosis, no halitosis]: WNL.
Neck [supple, thyroroid non-palpable, no enlarged cervical lymph nodes palpable]: WNL.
Extremities – [upper/lower extremities free of edema, cyanosis, varicosities, cellulitis, pedal pulses 2+]: WNL.
Gross musculoskeletal deformities: WNL.

There are no contraindications to chiropractic care.

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Segmental and somatic dysfunction of cervical region (M99.01)
2. Segmental and somatic dysfunction of thoracic region (M99.02)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Cervicalgia (M54.2)
5. Pain in thoracic spine (M54.6)
6. Muscle spasm of back (M62.830)
7. Low back pain, unspecified (M54.50)
8. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
11. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
12. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
13. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
14. Tension-type headache, unspecified, not intractable (G44.209)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

LONG-TERM GOALS

- Decrease pain to pre-injury status.
- Restore tolerance to normal activities of daily living.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

There is no past medical history that would contribute to the current exam findings.

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 2 stages of care; Acute and Rehabilitative.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 3 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 3 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 3 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 3 weeks.
- 99213 Re-evaluations will be performed once every 9 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 3 times per week for 3 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 3 times per week for 3 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 100%. This will be provided 3 times per week for 3 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 3 times per week for 3 weeks.
- 99213 Re-evaluations will be performed once every 9 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting was provided specifically to OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T10; T11; T12; L1; L2; L3; L4; L5, to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to

reduce pain reported at initial encounter by 50%.

- Diagnostic Tests: X-ray cervical 4-5 views (72050) was provided.
- Diagnostic Tests: X-ray thoracic 2 views (72070) was provided.
- Diagnostic Tests: X-ray lumbosacral 2-3 views (72100) was provided.
- New Patient evaluation and management service, 99203, was provided today.

The patient presents today for a New Patient Exam. The course of care and treatment plan was discussed with the patient. The patient had an opportunity to ask questions and a treatment plan was given to the patient which will be 3 times per week for 3 weeks.

Patient received care without incident.

(Electronic Signature)
Provider Name: Angela Alvarez, DC
Date: 06/11/2024 12:06 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Hijarah Hameen
Date of Birth: 6/24/2001

Provider: Samatha Ryder, DC
Date of Service: 6/13/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: **From 8 to 7/10 (10% decrease)**
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Left scapula, Right scapula, Upper back

- Aching pain: 7/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Mid back

- Aching pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Low back

- Aching pain: 5/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T10; T11; T12; L1; L2; L3; L4; L5**.**

Tenderness

Left Side	Right Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Segmental and somatic dysfunction of cervical region (M99.01)
2. Segmental and somatic dysfunction of thoracic region (M99.02)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Cervicalgia (M54.2)
5. Pain in thoracic spine (M54.6)
6. Muscle spasm of back (M62.830)
7. Low back pain, unspecified (M54.50)
8. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
11. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
12. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
13. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
14. Tension-type headache, unspecified, not intractable (G44.209)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, **diversified technique** was provided specifically to C2; C6; C7; T5; T6; T7; T8; L3; L4; L5, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- **Therapeutic Exercise (Ea. 15 Min) (97110) consisting of cervical active therapeutic movements was provided specifically to for 10 minutes (1 unit), to reduce pain**

reported at initial encounter by 25%.

Review x-rays with the patient. The patient had an opportunity to ask questions and **understand the findings**. **Due to positive findings on those X-rays**, the patient will be **referred** for **an MRI of the Cervical and Lumbar spine for further assessment**.

Patient received care without incident.

(Electronic Signature)
Provider Name: Samatha Ryder, DC
Date: 06/13/2024 10:25 AM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Hijarah Hameen
Date of Birth: 6/24/2001

Provider: Samatha Ryder, DC
Date of Service: 6/14/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Left scapula, Right scapula, Upper back

- Aching pain: 7/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Mid back

- Aching pain: 7/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Low back

- Aching pain: 5/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication and **OTC's**.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T10; T11; T12; L1; L2; L3; L4; L5**.**

Tenderness

Left Side	Right Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Segmental and somatic dysfunction of cervical region (M99.01)
2. Segmental and somatic dysfunction of thoracic region (M99.02)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Cervicalgia (M54.2)
5. Pain in thoracic spine (M54.6)
6. Muscle spasm of back (M62.830)
7. Low back pain, unspecified (M54.50)
8. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
11. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
12. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
13. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
14. Tension-type headache, unspecified, not intractable (G44.209)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to **C1**; **C5**; **T4**; T5; T6; T7; **T12**; **L1**; **L2**; L5, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Therapeutic Exercise (Ea. 15 Min) (97110) consisting of cervical active therapeutic movements was provided specifically to for 10 minutes (1 unit), to reduce pain

reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Samatha Ryder, DC
Date: 06/14/2024 09:54 AM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Hijarah Hameen
Date of Birth: 6/24/2001

Provider: Samatha Ryder, DC
Date of Service: 6/17/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: **From 7 to 5/10 (20% decrease)**
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Left scapula, Right scapula, Upper back

- Aching pain: **From 7 to 5/10 (20% decrease)**
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Mid back

- Aching pain: **From 7 to 5/10 (20% decrease)**
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Low back

- Aching pain: 5/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication and OTC's.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T10; T11; T12; L1; L2; L3; L4; L5**.**

Tenderness

Left Side	Right Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Segmental and somatic dysfunction of cervical region (M99.01)
2. Segmental and somatic dysfunction of thoracic region (M99.02)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Cervicalgia (M54.2)
5. Pain in thoracic spine (M54.6)
6. Muscle spasm of back (M62.830)
7. Low back pain, unspecified (M54.50)
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10. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
11. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
12. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
13. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
14. Tension-type headache, unspecified, not intractable (G44.209)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)

LONG-TERM GOALS

- Decrease pain to pre-injury status: **Progressed from 0% to 20% of goal achieved.**

Patient is **showing slight improvement** since the prior visit, She is **progressing as anticipated**. Intensity is **slightly-decreased** since the prior visit.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to **C5**; **C6**; **T3**; T4; T5; L1; L2; **L3**, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided

specifically to , to reduce pain reported at initial encounter by 50%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Therapeutic Exercise (Ea. 15 Min) (97110) consisting of cervical active therapeutic movements was provided specifically to for 10 minutes (1 unit), to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Samatha Ryder, DC
Date: 06/17/2024 10:01 AM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Hijarah Hameen
Date of Birth: 6/24/2001

Provider: Samatha Ryder, DC
Date of Service: 6/18/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 5/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Left scapula, Right scapula, Upper back

- Aching pain: 5/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Mid back

- Aching pain: 5/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Low back

- Aching pain: 5/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication and OTC's.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T10; T11; T12; L1; L2; L3; L4; L5.

Tenderness

Left Side	Right Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Segmental and somatic dysfunction of cervical region (M99.01)
2. Segmental and somatic dysfunction of thoracic region (M99.02)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Cervicalgia (M54.2)
5. Pain in thoracic spine (M54.6)
6. Muscle spasm of back (M62.830)
7. Low back pain, unspecified (M54.50)
8. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
11. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
12. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
13. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
14. Tension-type headache, unspecified, not intractable (G44.209)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. **Segmental and somatic dysfunction of pelvic region (M99.05)**

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C5; T4; L2; L3; **Pelvis**, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Therapeutic Exercise (Ea. 15 Min) (97110) consisting of cervical active therapeutic

movements was provided specifically to for 10 minutes (1 unit), to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Samatha Ryder, DC
Date: 06/18/2024 11:19 AM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Hijarah Hameen
Date of Birth: 6/24/2001

Provider: Samatha Ryder, DC
Date of Service: 6/19/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: **From 5 to 4/10 (10% decrease)**
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Left scapula, Right scapula, Upper back

- Aching pain: **From 5 to 4/10 (10% decrease)**
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Mid back

- Aching pain: **From 5 to 4/10 (10% decrease)**
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Low back

- Aching pain: **From 5 to 4/10 (10% decrease)**
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication and OTC's.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T10; T11; T12; L1; L2; L3; L4; L5**.**

Tenderness

Left Side	Right Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Segmental and somatic dysfunction of cervical region (M99.01)
2. Segmental and somatic dysfunction of thoracic region (M99.02)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Cervicalgia (M54.2)
5. Pain in thoracic spine (M54.6)
6. Muscle spasm of back (M62.830)
7. Low back pain, unspecified (M54.50)
8. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
11. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
12. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
13. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
14. Tension-type headache, unspecified, not intractable (G44.209)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Segmental and somatic dysfunction of pelvic region (M99.05)

Patient is **showing slight improvement** since the prior visit, She is **progressing as anticipated**. Intensity is **slightly-decreased** since the prior visit.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to **C2**; **C5**; T4; **T10**; **L1**; L2; L3; **L4**; **L5**; Pelvis, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain

reported at initial encounter by 50%.

- Therapeutic Exercise (Ea. 15 Min) (97110) consisting of cervical active therapeutic movements was provided specifically to for 10 minutes (1 unit), to reduce pain reported at initial encounter by 25%.
- **Established Patient evaluation and management service, 99212, was provided today.**

MRI Results were reviewed today with the patient. The patient had the opportunity to ask questions and understand the findings. Due to positive findings, the patient will be referred to a specialist for further assessment.

Patient received care without incident.

(Electronic Signature)
Provider Name: Samatha Ryder, DC
Date: 06/19/2024 10:44 AM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Hijarah Hameen
Date of Birth: 6/24/2001

Provider: Samatha Ryder, DC
Date of Service: 6/20/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 4/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Left scapula, Right scapula, Upper back

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Mid back

- Aching pain: 4/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Low back

- Aching pain: 4/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication and OTC's.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T10; T11; T12; L1; L2; L3; L4; L5.

Tenderness

Left Side	Right Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Segmental and somatic dysfunction of cervical region (M99.01)
2. Segmental and somatic dysfunction of thoracic region (M99.02)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Cervicalgia (M54.2)
5. Pain in thoracic spine (M54.6)
6. Muscle spasm of back (M62.830)
7. Low back pain, unspecified (M54.50)
8. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
11. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
12. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
13. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
14. Tension-type headache, unspecified, not intractable (G44.209)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Segmental and somatic dysfunction of pelvic region (M99.05)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to **T1**; **T8**; **T12**; L1; L2; Pelvis, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Therapeutic Exercise (Ea. 15 Min) (97110) consisting of cervical active therapeutic

movements was provided specifically to for 10 minutes (1 unit), to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Samatha Ryder, DC
Date: 06/20/2024 01:14 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Hijarah Hameen
Date of Birth: 6/24/2001

Provider: Samatha Ryder, DC
Date of Service: 6/24/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 4/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Left scapula, Right scapula, Upper back

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Mid back

- Aching pain: 4/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Low back

- Aching pain: 4/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication and OTC's.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T10; T11; T12; L1; L2; L3; L4; L5.

Tenderness

Left Side	Right Side

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

- Cervical Spine (Moderate)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

Right Side

- Cervical Spine (Moderate-Severe)
- Thoracic Spine (Moderate-Severe)
- Lower Back (Moderate-Severe)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Segmental and somatic dysfunction of cervical region (M99.01)
2. Segmental and somatic dysfunction of thoracic region (M99.02)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Cervicalgia (M54.2)
5. Pain in thoracic spine (M54.6)
6. Muscle spasm of back (M62.830)
7. Low back pain, unspecified (M54.50)
8. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
11. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
12. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
13. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
14. Tension-type headache, unspecified, not intractable (G44.209)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Segmental and somatic dysfunction of pelvic region (M99.05)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to **C4**; **T3**; **T6**; T8; L2, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Therapeutic Exercise (Ea. 15 Min) (97110) consisting of cervical active therapeutic

movements was provided specifically to for 10 minutes (1 unit), to reduce pain reported at initial encounter by 25%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Samatha Ryder, DC
Date: 06/24/2024 04:01 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Exam

Patient Name: Hijarah Hameen
Date of Birth: 6/24/2001

Provider: Samatha Ryder, DC
Date of Service: 6/25/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 4/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Left scapula, Right scapula, Upper back

- Aching pain: 4/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Mid back

- Aching pain: 4/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Low back

- Aching pain: 4/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication and OTC's.

OBJECTIVE

VITALS
Height: 63 in Weight: 146.2 lbs BMI: 25.9
Temperature: 97.3°F Pulse rate: 92 BPM (relaxed)
Blood Pressure (seated): right -N/A left-99/58 mmHg

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T10; T11; T12; L1; L2; L3; L4; L5; **T9; Pelvis.**

Tenderness

Left Side

- Cervical Spine **(decreased 10-20% to mild)**
- Thoracic Spine **(decreased 10-20% to mild-moderate)**
- Lower Back **(decreased 10-20% to mild-moderate)**

Right Side

- Cervical Spine **(decreased 10-20% to mild)**
- Thoracic Spine **(decreased 10-20% to mild-moderate)**
- Lower Back **(decreased 10-20% to mild-moderate)**

Spasm/Hypertonicity

Left Side

- Cervical Spine **(decreased 10-20% to mild-moderate)**
- Thoracic Spine **(decreased 10-20% to mild-moderate)**
- Lower Back **(decreased 10-20% to mild-moderate)**

Right Side

- Cervical Spine **(decreased 10-20% to mild-moderate)**
- Thoracic Spine **(decreased 10-20% to mild-moderate)**
- Lower Back **(decreased 10-20% to mild-moderate)**

GAIT

Ambulation: **Normal**.

Gait: **Normal**.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Test	Left	Right	Comment
Spinal percussion test-cervical[1]	Positive	Positive	
Spinal percussion test-thoracic[2]	Positive	Positive	

[1]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

[2]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

Maximal foraminal compression test, Shoulder depression test, Soto-Hall, Schepelmann's sign, Demianoff sign test, Milgram's test and Spinal percussion test-lumbar are within normal limits.

REFLEXES

Biceps and Brachioradialis are within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	**40° (80% of normal, increased by 60%)**	Pain	
Extension	**50° (83% of normal,**	Pain	

	increased by 42%)		
Left lateral flexion	**40° (88% of normal, increased by 33%)**	Pain	
Right lateral flexion	**40° (88% of normal, increased by 60%)**	Pain	
Left rotation	**70° (87% of normal, increased by 75%)**	Pain	
Right rotation	**70° (87% of normal, increased by 40%)**	Pain	

Lumbar ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	**50° (83% of normal, increased by 66%)**	Pain	
Extension	**20° (80% of normal, increased by 100%)**	Pain	
Left lateral flexion	20° (80% of normal)	Pain	
Right ateral flexion	**20° (80% of normal, increased by 33%)**	Pain	
Left rotation	**25° (83% of normal, increased by 66%)**	Pain	
Right rotation	**25° (83% of normal, increased by 25%)**	Pain	

DERMATOMES

Posterior

Left-Cervical, Right-Cervical, Left-Lumbar and Right-Lumbar are within normal limits.

PHYSICAL EXAMINATION

General [well nourished, no acute distress]: WNL.
Skin [no skin sensitivity reported, free of lesions, rashes, eczema, upper/lower extremities]: WNL.
Head [eyes – symmetric appearance, conjunctiva white, no discharge, erythema, lid lag or swelling]: WNL.
Ears [no discharge or deformities noted to external ears]: WNL.
Nose [no sinus tenderness, epistaxis, or discharge]: WNL.
Oropharynx [lips free of lesions, cyanosis, no halitosis]: WNL.
Neck [supple, thyroroid non-palpable, no enlarged cervical lymph nodes palpable]: WNL.
Extremities – [upper/lower extremities free of edema, cyanosis, varicosities, cellulitis, pedal pulses 2+]: WNL.
Gross musculoskeletal deformities: WNL.

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Segmental and somatic dysfunction of cervical region (M99.01)
2. Segmental and somatic dysfunction of thoracic region (M99.02)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Cervicalgia (M54.2)

5. Pain in thoracic spine (M54.6)
6. Muscle spasm of back (M62.830)
7. Low back pain, unspecified (M54.50)
8. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
11. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
12. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
13. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
14. Tension-type headache, unspecified, not intractable (G44.209)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Segmental and somatic dysfunction of pelvic region (M99.05)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to **OCC**; **C7**; **T3**; T4; T5; **T6**; **L2**; **Pelvis**, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Therapeutic Exercise (Ea. 15 Min) (97110) consisting of cervical active therapeutic movements was provided specifically to for 10 minutes (1 unit), to reduce pain reported at initial encounter by 25%.
- **Established Patient evaluation and management service, 99213, was provided today.**

Reexam today. The new care plan will be 3 times per week for 3 weeks.

Patient received care without incident.

(Electronic Signature)
Provider Name: Samatha Ryder, DC
Date: 06/25/2024 04:37 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Hijarah Hameen
Date of Birth: 6/24/2001

Provider: Samatha Ryder, DC
Date of Service: 6/27/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: **From 4 to 3/10 (10% decrease)**
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Left scapula, Right scapula, Upper back

- Aching pain: **From 4 to 3/10 (10% decrease)**
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Mid back

- Aching pain: **From 4 to 3/10 (10% decrease)**
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Low back

- Aching pain: **From 4 to 3/10 (10% decrease)**
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication and OTC's.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5; Pelvis**.**

Tenderness

Left Side	Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Cervical Spine (Mild)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Segmental and somatic dysfunction of cervical region (M99.01)
2. Segmental and somatic dysfunction of thoracic region (M99.02)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Cervicalgia (M54.2)
5. Pain in thoracic spine (M54.6)
6. Muscle spasm of back (M62.830)
7. Low back pain, unspecified (M54.50)
8. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
11. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
12. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
13. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
14. Tension-type headache, unspecified, not intractable (G44.209)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Segmental and somatic dysfunction of pelvic region (M99.05)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to OCC; C7; L2; Pelvis, to **reduce pain reported at initial encounter by 100%**.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to **reduce pain reported at initial encounter by 100%**.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to **Back**, **Neck**, to **reduce pain reported at initial encounter by 100%**.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to **reduce pain reported at initial encounter by 100%**.

Patient received care without incident.

(Electronic Signature)
Provider Name: Samatha Ryder, DC
Date: 06/27/2024 01:08 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Hijarah Hameen
Date of Birth: 6/24/2001

Provider: Samatha Ryder, DC
Date of Service: 6/28/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 3/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Left scapula, Right scapula, Upper back

- Aching pain: 3/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Mid back

- Aching pain: 3/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Low back

- Aching pain: 3/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication and OTC's.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5; Pelvis.

Tenderness

Left Side	Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Cervical Spine (Mild)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Segmental and somatic dysfunction of cervical region (M99.01)
2. Segmental and somatic dysfunction of thoracic region (M99.02)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Cervicalgia (M54.2)
5. Pain in thoracic spine (M54.6)
6. Muscle spasm of back (M62.830)
7. Low back pain, unspecified (M54.50)
8. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
11. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
12. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
13. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
14. Tension-type headache, unspecified, not intractable (G44.209)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Segmental and somatic dysfunction of pelvic region (M99.05)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to **C7**; **T4**; **T8**; L2; Pelvis, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.
- **Therapeutic Exercise (Ea. 15 Min) (97110) consisting of cervical active therapeutic movements, lumbar active therapeutic movements was provided specifically to Neck, Back for 10 minutes (1 unit), to reduce pain reported at initial encounter by 50%.**

Patient received care without incident.

(Electronic Signature)
Provider Name: Samatha Ryder, DC
Date: 06/28/2024 01:00 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Hijarah Hameen
Date of Birth: 6/24/2001

Provider: Samatha Ryder, DC
Date of Service: 7/1/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 3/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Left scapula, Right scapula, Upper back

- Aching pain: 3/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Mid back

- Aching pain: 3/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Low back

- Aching pain: 3/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication and OTC's.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5; Pelvis.

Tenderness

Left Side	Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Cervical Spine (Mild)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Segmental and somatic dysfunction of cervical region (M99.01)
2. Segmental and somatic dysfunction of thoracic region (M99.02)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Cervicalgia (M54.2)
5. Pain in thoracic spine (M54.6)
6. Muscle spasm of back (M62.830)
7. Low back pain, unspecified (M54.50)
8. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
11. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
12. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
13. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
14. Tension-type headache, unspecified, not intractable (G44.209)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Segmental and somatic dysfunction of pelvic region (M99.05)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to **OCC**; C7; T4; L2; Pelvis, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Therapeutic Exercise (Ea. 15 Min) (97110) consisting of cervical active therapeutic movements, lumbar active therapeutic movements was provided specifically to Neck, Back for 10 minutes (1 unit), to reduce pain reported at initial encounter by 50%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Samatha Ryder, DC
Date: 07/01/2024 01:01 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Hijarah Hameen
Date of Birth: 6/24/2001

Provider: Samatha Ryder, DC
Date of Service: 7/2/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 3/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Left scapula, Right scapula, Upper back

- Aching pain: 3/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Mid back

- Aching pain: 3/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Low back

- Aching pain: 3/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication and OTC's.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5; Pelvis**.**

Tenderness

Left Side	Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

- Cervical Spine (Mild)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back (Mild-Moderate)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Segmental and somatic dysfunction of cervical region (M99.01)
2. Segmental and somatic dysfunction of thoracic region (M99.02)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Cervicalgia (M54.2)
5. Pain in thoracic spine (M54.6)
6. Muscle spasm of back (M62.830)
7. Low back pain, unspecified (M54.50)
8. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
11. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
12. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
13. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
14. Tension-type headache, unspecified, not intractable (G44.209)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Segmental and somatic dysfunction of pelvic region (M99.05)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to **C3**; **C6**; T4; L2; Pelvis, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Therapeutic Exercise (Ea. 15 Min) (97110) consisting of cervical active therapeutic movements, lumbar active therapeutic movements was provided specifically to Neck, Back for 10 minutes (1 unit), to reduce pain reported at initial encounter by 50%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Samatha Ryder, DC
Date: 07/02/2024 11:42 AM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Hijarah Hameen
Date of Birth: 6/24/2001

Provider: Samatha Ryder, DC
Date of Service: 7/3/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 3/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Left scapula, Right scapula, Upper back

- Aching pain: 3/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Mid back

- Aching pain: 3/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Low back

- Aching pain: 3/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication and OTC's.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5; Pelvis.

Tenderness

Left Side	Right Side

- Thoracic Spine **(decreased 5-10% to mild)**
- Lower Back **(decreased 5-10% to mild)**
- Cervical Spine (Mild)

- Thoracic Spine **(decreased 5-10% to mild)**
- Lower Back **(decreased 5-10% to mild)**
- Cervical Spine (Mild)

Spasm/Hypertonicity

Left Side

- Cervical Spine **(decreased 5-10% to mild)**
- Thoracic Spine **(decreased 5-10% to mild)**
- Lower Back **(decreased 5-10% to mild)**

Right Side

- Cervical Spine **(decreased 5-10% to mild)**
- Thoracic Spine **(decreased 5-10% to mild)**
- Lower Back **(decreased 5-10% to mild)**

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Segmental and somatic dysfunction of cervical region (M99.01)
2. Segmental and somatic dysfunction of thoracic region (M99.02)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Cervicalgia (M54.2)
5. Pain in thoracic spine (M54.6)
6. Muscle spasm of back (M62.830)
7. Low back pain, unspecified (M54.50)
8. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
11. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
12. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
13. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
14. Tension-type headache, unspecified, not intractable (G44.209)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Segmental and somatic dysfunction of pelvic region (M99.05)

Patient is **showing slight improvement** since the prior visit, She is **progressing as anticipated**. Intensity is **slightly-decreased** since the prior visit.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C3; **T1**; **T10**; L2; Pelvis, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.

- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Therapeutic Exercise (Ea. 15 Min) (97110) consisting of cervical active therapeutic movements, lumbar active therapeutic movements was provided specifically to Neck, Back for 10 minutes (1 unit), to reduce pain reported at initial encounter by 50%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Samatha Ryder, DC
Date: 07/03/2024 04:15 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Hijarah Hameen
Date of Birth: 6/24/2001

Provider: Samatha Ryder, DC
Date of Service: 7/8/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: **From 3 to 2/10 (10% decrease)**
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Left scapula, Right scapula, Upper back

- Aching pain: **From 3 to 2/10 (10% decrease)**
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Mid back

- Aching pain: **From 3 to 2/10 (10% decrease)**
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Low back

- Aching pain: **From 3 to 2/10 (10% decrease)**
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication and OTC's.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8; T9; T10; T11; T12; L1; L2; L3; L4; L5; Pelvis**.**

Tenderness

Left Side	Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Segmental and somatic dysfunction of cervical region (M99.01)
2. Segmental and somatic dysfunction of thoracic region (M99.02)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Cervicalgia (M54.2)
5. Pain in thoracic spine (M54.6)
6. Muscle spasm of back (M62.830)
7. Low back pain, unspecified (M54.50)
8. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
11. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
12. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
13. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
14. Tension-type headache, unspecified, not intractable (G44.209)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Segmental and somatic dysfunction of pelvic region (M99.05)

Patient is **showing slight improvement** since the prior visit, She is **progressing as anticipated**. Intensity is **slightly-decreased** since the prior visit.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C3; T1; T10; L2; Pelvis, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.

- Therapeutic Exercise (Ea. 15 Min) (97110) consisting of cervical active therapeutic movements, lumbar active therapeutic movements was provided specifically to Neck, Back for 10 minutes (1 unit), to reduce pain reported at initial encounter by 50%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Samatha Ryder, DC
Date: 07/08/2024 01:05 PM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Hijarah Hameen
Date of Birth: 6/24/2001

Provider: Samatha Ryder, DC
Date of Service: 7/9/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: 2/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Left scapula, Right scapula, Upper back

- Aching pain: 2/10
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Mid back

- Aching pain: 2/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Low back

- Aching pain: 2/10
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication and OTC's.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: OCC; C6; C7; T5; T6; L1; L2; Pelvis**. No findings: C1; C2; C3; C4; C5; T1; T2; T3; T4; T7; T8; T9; T10; T11; T12; L3; L4; L5**.

Tenderness

Left Side	Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Mild)
- Lower Back (Mild)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Segmental and somatic dysfunction of cervical region (M99.01)
2. Segmental and somatic dysfunction of thoracic region (M99.02)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Cervicalgia (M54.2)
5. Pain in thoracic spine (M54.6)
6. Muscle spasm of back (M62.830)
7. Low back pain, unspecified (M54.50)
8. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
11. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
12. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
13. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
14. Tension-type headache, unspecified, not intractable (G44.209)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Segmental and somatic dysfunction of pelvic region (M99.05)

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to **OCC**; **C6**; **T5**; L2; Pelvis, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Therapeutic Exercise (Ea. 15 Min) (97110) consisting of cervical active therapeutic movements, lumbar active therapeutic movements was provided specifically to Neck, Back for 10 minutes (1 unit), to reduce pain reported at initial encounter by 50%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Samatha Ryder, DC
Date: 07/09/2024 10:29 AM

Iceberg Health Marietta
62 Shawnee Trail SE
Marietta, GA 30067-7316
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Exam

Patient Name: Hijarah Hameen
Date of Birth: 6/24/2001

Provider: Samatha Ryder, DC
Date of Service: 7/10/2024

SUBJECTIVE

PRIMARY COMPLAINT
Neck

- Aching pain: **From 2 to 1/10 (10% decrease)**
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

SECOND COMPLAINT
Left scapula, Right scapula, Upper back

- Aching pain: **From 2 to 1/10 (10% decrease)**
  Described as frequent (up to 75% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

THIRD COMPLAINT
Mid back

- Aching pain: **From 2 to 1/10 (10% decrease)**
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication.

FOURTH COMPLAINT
Low back

- Aching pain: **From 2 to 1/10 (10% decrease)**
  Described as frequent to constant (up to between 75% and 100% of day) of moderate to severe intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Prescribed Pain Medication and OTC's.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: OCC; C6; C7; T5; T6; L1; L2; Pelvis**.**

Tenderness

Left Side

- Cervical Spine **(decreased 5-10% to minimal)**
- Thoracic Spine **(decreased 5-10% to minimal)**

Right Side

- Cervical Spine **(decreased 5-10% to minimal)**
- Thoracic Spine **(decreased 5-10% to minimal)**

- Lower Back **(decreased 5-10% to minimal)**
- Lower Back **(decreased 5-10% to minimal)**

Spasm/Hypertonicity

Left Side

- Cervical Spine **(decreased 5-10% to minimal)**
- Thoracic Spine **(decreased 5-10% to minimal)**
- Lower Back **(decreased 5-10% to minimal)**

Right Side

- Cervical Spine **(decreased 5-10% to minimal)**
- Thoracic Spine **(decreased 5-10% to minimal)**
- Lower Back **(decreased 5-10% to minimal)**

GAIT

Ambulation: Normal.
Gait: Normal.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Maximal foraminal compression test, Shoulder depression test, Soto-Hall, **Spinal percussion test-cervical**, Schepelmann's sign, **Spinal percussion test-thoracic**, Demianoff sign test, Milgram's test and Spinal percussion test-lumbar are within normal limits.

REFLEXES

Biceps and Brachioradialis are within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	**45° (90% of normal, increased by 12%)**		
Extension	**55° (91% of normal, increased by 10%)**		
Left rotation	**75° (93% of normal, increased by 7%)**		
Right rotation	**75° (93% of normal, increased by 7%)**		

Left lateral flexion and **Right lateral flexion** are within normal limits.

Lumbar ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	**55° (91% of normal, increased by 10%)**		

Extension, **Left lateral flexion**, **Right ateral flexion**, **Left rotation** and **Right rotation** are within normal limits.

DERMATOMES

Posterior

Left-Cervical, Right-Cervical, Left-Lumbar and Right-Lumbar are within normal limits.

PHYSICAL EXAMINATION

General [well nourished, no acute distress]: WNL.
Skin [no skin sensitivity reported, free of lesions, rashes, eczema, upper/lower extremities]: WNL.
Head [eyes – symmetric appearance, conjunctiva white, no discharge, erythema, lid lag or swelling]: WNL.
Ears [no discharge or deformities noted to external ears]: WNL.
Nose [no sinus tenderness, epistaxis, or discharge]: WNL.

Oropharynx [lips free of lesions, cyanosis, no halitosis]: WNL.
Neck [supple, thyroroid non-palpable, no enlarged cervical lymph nodes palpable]: WNL.
Extremities – [upper/lower extremities free of edema, cyanosis, varicosities, cellulitis, pedal pulses 2+]: WNL.
Gross musculoskeletal deformities: WNL.

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Segmental and somatic dysfunction of cervical region (M99.01)
2. Segmental and somatic dysfunction of thoracic region (M99.02)
3. Segmental and somatic dysfunction of lumbar region (M99.03)
4. Cervicalgia (M54.2)
5. Pain in thoracic spine (M54.6)
6. Muscle spasm of back (M62.830)
7. Low back pain, unspecified (M54.50)
8. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
9. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
10. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
11. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
12. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
13. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
14. Tension-type headache, unspecified, not intractable (G44.209)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Segmental and somatic dysfunction of pelvic region (M99.05)

LONG-TERM GOALS

- Decrease pain to pre-injury status: **Progressed from 20% to 90% of goal achieved.**
- Restore tolerance to normal activities of daily living: **Progressed from 0% to 90% of goal achieved.**

Patient is **showing slight improvement** since the prior visit, She is **progressing as anticipated**. Intensity is **slightly-decreased** since the prior visit.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C6; T5; L2; Pelvis, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Back, Neck, to reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Therapeutic Exercise (Ea. 15 Min) (97110) consisting of cervical active therapeutic movements, lumbar active therapeutic movements was provided specifically to Neck, Back for 10 minutes (1 unit), to reduce pain reported at initial encounter by 50%.
- **Established Patient evaluation and management service, 99213, was provided today.**

Final exam today. The patient has reached maximum medical improvement in this office. The patient will continue with home care instructions if symptoms persist.

Patient received care without incident.

(Electronic Signature)
Provider Name: Samatha Ryder, DC
Date: 07/10/2024 10:39 AM